This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 15 OCTOBER 2026

Patient Group Direction

for the supply of Daridorexant 50 mg or 25 mg film-coated tablets (QUVIVIQ, a black triangle medicine) for the treatment of Long-Term Insomnia

Long-Term Insomnia

Adults aged 18 to 75 years at the first supply.

Patient Group Direction, version 001, issued 15 October 2026. Get Real Health Limited. Daridorexant 50 mg and 25 mg film-coated tablets (QUVIVIQ), a black triangle medicine. Adults aged 18 to 75 with long-term insomnia meeting NICE TA922. An initiation course of up to three 30-tablet supplies, then a prescriber. Pharmacists only.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

This Patient Group Direction, issued by Get Real Health Limited, authorises the supply of daridorexant to an adult aged 18 to 75 with long-term insomnia (symptoms on 3 or more nights a week for at least 3 months, with a considerable effect on daytime functioning) for whom cognitive behavioural therapy for insomnia (CBT-I) has been tried and has not worked, or is not available, or is unsuitable, as NICE TA922 recommends. Supply is by a registered pharmacist in a community pharmacy, face to face on the premises, without a prescription, as a private service. It is an initiation course only: any continuation is for a prescriber. QUVIVIQ is a BLACK TRIANGLE MEDICINE: it is subject to additional monitoring and every suspected adverse reaction is reported to the MHRA Yellow Card scheme.

What it authorises

- Supply 1: 30 tablets of 50 mg once nightly, or 25 mg once nightly only for a patient who regularly takes a moderate CYP3A4 inhibitor (Appendix 4), after an assessment that includes a sleep diary of 1 to 2 weeks.
- Supply 2: a further 30 tablets after an early safety review at supply visit 2, at the dose set under Dose and frequency (50 mg, or 25 mg while a regular moderate CYP3A4 inhibitor is taken).
- Supply 3: a further 30 tablets, the last under this PGD, only where the effectiveness assessment at supply visit 3 shows an adequate response as defined under Dose and frequency, with a handover letter to the GP or another prescriber.
- MAXIMUM UNDER THIS PGD: three supplies of 30 tablets (90 tablets, about 13 weeks of nightly treatment), once per patient, counted by supply number from the first supply and never reset. Any treatment after supply 3 is for a prescriber.

Who may work under it

Registered pharmacists only. The insomnia assessment, the screening questionnaires, the interaction and Summary Care Record checks, the brief CBT-I advice and the effectiveness assessment are not delegated to pharmacy technicians.

What it does not cover

- Anyone under 18, or aged 76 or over at the first supply. The SmPC advises caution over 75 because data are limited, and there are no data over 85.

- Short-term or acute insomnia (under 3 months), insomnia on fewer than 3 nights a week, or poor sleep without a considerable effect on daytime functioning.
- Anyone for whom NHS CBT-I (face to face, group or digital, including free NHS digital CBT-I) is available and suitable and has not been tried. A preference not to try CBT-I does not make it unsuitable, and the patient's choice about paying for a private programme does not change whether NHS CBT-I is available (see the inclusion criteria).
- Continuation beyond three supplies, a second course, or a patient already taking daridorexant on prescription. These are for a prescriber.
- Insomnia that may be caused by another sleep disorder (sleep apnoea, narcolepsy, restless legs, a circadian rhythm disorder or shift work), by excessive daytime sleepiness, or by an untreated physical or mental health condition, medicine or substance. Refer.
- Anyone taking a strong CYP3A4 inhibitor, a moderate or strong CYP3A4 inducer, another hypnotic or a medicine that causes drowsiness taken regularly and continuing, or a medicine listed in Appendix 4 as excluded.
- Anyone who must drive or do safety-critical work in the morning and cannot keep at least 9 hours between a dose and starting; the sole night-time carer of an infant or a dependent adult.
- Pregnancy, planned pregnancy or breastfeeding.
- Doses other than 50 mg or 25 mg once nightly, use as and when needed, or any dose taken during the night.
- Remote or telephone consultations, supply by a pharmacy technician, or a patient not registered with a GP.

Summary of the governing guidance for daridorexant in long-term insomnia

QUVIVIQ 25 mg and 50 mg film-coated tablets SmPC (Idorsia, PLGB 48711/0002 and PLGB 48711/0003, text revised 12 May 2025, black triangle), medicines.org.uk; NICE Technology Appraisal TA922, Daridorexant for treating long-term insomnia, published 18 October 2023; Scottish Medicines Consortium SMC2611, daridorexant (Quviviq), advice 8 March 2024, published 8 April 2024; NICE Medicines Practice Guideline MPG2, Patient Group Directions; NICE Clinical Knowledge Summary, Insomnia; NHS Grampian (Sleepio in Scotland) and NHS Wales SilverCloud self-referral guide. Summarised 15 October 2026.

The condition

Insomnia is difficulty getting to sleep, difficulty staying asleep, early waking or non-restorative sleep, despite adequate opportunity for sleep, that impairs daytime functioning (poor concentration, mood disturbance, fatigue). Sleep disturbance without daytime impairment is not insomnia disorder. Symptoms on at least 3 nights a week for 3 months or more is long-term (chronic) insomnia (NICE CKS).

Before any treatment, identify and manage what may be causing or maintaining it: medical conditions (for example chronic pain, hot flushes, obstructive sleep apnoea), mental health conditions (for example depression), medicines that disturb sleep (for example stimulants, some antidepressants, corticosteroids, diuretics), alcohol, caffeine and other substances, and circadian or other sleep disorders (NICE CKS).

Sleep hygiene advice is offered first. Cognitive behavioural therapy for insomnia (CBT-I), face to face or digital, is the standard first treatment for long-term insomnia after sleep hygiene advice (NICE TA922, NICE CKS, SMC2611). NICE medical technologies guidance MTG70 recommends the digital CBT-I programme Sleepio in primary care for people who would otherwise be offered sleep hygiene advice or sleeping pills.

NHS digital CBT-I access differs across the UK. Sleepio is free to anyone living in Scotland, funded by the Scottish Government (NHS Grampian). In Wales, SilverCloud programmes, including Space for Sleep (based on CBT-I), are free by self-referral for people aged 16 and over living in Wales (NHS Wales SilverCloud guide). In England, access depends on local commissioning and NHS Talking Therapies. NICE TA922 notes that access to CBT-I varies across the UK.

NICE TA922 and SMC2611: who daridorexant is for

NICE TA922 recommends daridorexant for insomnia in adults with symptoms on 3 or more nights a week for at least 3 months, whose daytime functioning is considerably affected, ONLY IF cognitive behavioural therapy for insomnia has been tried but not worked, or is not available or is unsuitable.

Treatment should be as short as possible. It should be assessed within 3 months of starting and STOPPED in people whose long-term insomnia has not responded adequately. If it is continued, whether it is still working is assessed at regular intervals (TA922 1.2).

The committee noted trial evidence of benefit over placebo for up to 12 months; effects beyond 12 months are unknown.

SMC2611 accepts daridorexant for restricted use in NHSScotland on the same basis: patients who have failed CBT-I, or for whom CBT-I is unsuitable or unavailable. SMC noted that most trial patients had never received or been offered CBT-I, so the trials give evidence where CBT-I is unavailable or unsuitable but minimal evidence in people who have failed it. On sleep measures, the European regulator considered the 50 mg results clinically relevant and the 25 mg results questionable. Effects on daytime functioning appeared modest, and the European regulator noted that their clinical relevance is questionable. About 10 per cent more patients than placebo reached an Insomnia Severity Index below 10 at 3 months.

ISI CHANGE (SMC2611 Table 2.2, an exploratory outcome): mean fall from baseline at month 3 was 7.2 points on 50 mg, 6.0 on 25 mg and 5.4 on placebo in Study 301 (6.9 on 25 mg and 5.4 on placebo in Study 302). Clinical experts consulted by SMC noted that benzodiazepines, Z-drugs and melatonin are used off-label for prolonged periods in insomnia.

The licence (SmPC)

INDICATION: adults with insomnia characterised by symptoms present for at least 3 months and considerable impact on daytime functioning.

DOSE: 50 mg once a night, taken in the evening within 30 minutes before going to bed. Maximum 50 mg a day. 25 mg once a night with a moderate CYP3A4 inhibitor, in moderate hepatic impairment, and for some patients on clinical judgement. Not taken during the night if forgotten at bedtime. Taking it soon after a large meal may delay the effect. Grapefruit or grapefruit juice in the evening should be avoided. Can be stopped without tapering; no rebound insomnia was seen after 3 months.

DURATION: as short as possible; appropriateness of continued treatment assessed within 3 months and periodically thereafter. Clinical data cover up to 12 months of continuous treatment.

CONTRAINDICATIONS: hypersensitivity to daridorexant or an excipient; narcolepsy; concomitant strong CYP3A4 inhibitors (for example itraconazole, clarithromycin, ritonavir).

WARNINGS: caution in the elderly because of falls, and in those over 75 (limited data; no data over 85). CNS depressant effect: caution with other CNS depressants and alcohol, and about driving or hazardous activity until fully alert, especially in the first few days. Sleep paralysis, hypnagogic and hypnopompic hallucinations, and cataplexy-like symptoms (dual orexin receptor antagonist class) can occur, mainly in the first weeks: patients need further evaluation and discontinuation should be considered. Worsening of depression and suicidal thoughts have been reported with hypnotics in primarily depressed patients; caution with symptoms of depression and with psychiatric co-morbidity (limited data). Increased body sway on being woken 4 hours after a 25 or 50 mg dose was measured in healthy adults under 65 (SmPC 5.1). No worsening of sleep apnoea or oxygen saturation in mild to severe obstructive sleep apnoea or moderate COPD; not studied in severe COPD (FEV1 below 40 per cent predicted), where caution is needed. In an abuse liability study, 100 mg and 150 mg produced drug liking similar to zolpidem 30 mg; people with a history of alcohol or substance misuse need careful follow-up. No withdrawal syndrome on stopping. Not recommended in severe hepatic impairment.

DRIVING: in a simulator study, driving was impaired the morning after the first dose (9 hours after 50 mg) but not after 4 nights. About 9 hours should pass between taking a dose and driving or using machines.

PREGNANCY AND BREASTFEEDING: no human pregnancy data; use only if the clinical condition requires it. Excreted in breast milk in small amounts; a risk of excessive sleepiness in the breastfed infant cannot be excluded.

ADVERSE REACTIONS: common headache, somnolence, dizziness, fatigue, nausea; uncommon hypersensitivity (rash, urticaria), hallucinations, abnormal dreams and nightmares, sleepwalking (somnambulism), sleep paralysis. No dose relationship; the profile in older adults matched younger adults. The GB SmPC does not carry a separate complex sleep behaviour warning; sleepwalking is listed as an adverse reaction, and this PGD treats any complex sleep behaviour as a stop symptom.

BLACK TRIANGLE: subject to additional monitoring. Report every suspected adverse reaction to the MHRA Yellow Card scheme.

Interactions (SmPC section 4.5)

Daridorexant is about 89 per cent metabolised by CYP3A4. Strong CYP3A4 inhibitors: contraindicated. Moderate inhibitors (for example diltiazem, which raised exposure 2.4 times, erythromycin, ciprofloxacin, ciclosporin): 25 mg. Moderate or strong inducers (for example efavirenz, which cut exposure by 61 per cent): substantially reduced exposure and possible loss of effect.

At 50 mg daridorexant is a mild CYP3A4 inhibitor (midazolam exposure up 42 per cent) and a mild P-gp inhibitor (dabigatran exposure up 42 per cent): caution with narrow therapeutic index CYP3A4 substrates (for example high-dose simvastatin, tacrolimus) and P-gp substrates (for example digoxin). No effect on warfarin, rosuvastatin or hormonal contraceptives; no clinically relevant interaction with citalopram; acid-reducing medicines need no dose change.

Alcohol has additive effects on psychomotor performance. Other CNS depressants may need a dose change of either medicine on clinical evaluation.

Legal status and use under a PGD

QUVIVIQ is a prescription only medicine (emc). It is not a controlled drug in the UK: daridorexant is not named in the Misuse of Drugs Regulations 2001 or any amending instrument (legislation.gov.uk full-text search, 15 October 2026), and emc lists it as POM only. It is a Schedule IV controlled substance in the United States, which has no effect on UK status.

NICE MPG2: most supply should be on an individual, patient-specific basis, with PGDs reserved for limited situations in which they offer an advantage for patient care without compromising safety, with clear governance (1.1.1). A black triangle medicine may be included only when clearly justified by best clinical practice, with the black triangle status clearly shown (1.1.8). Doses of a medicine already in the patient's possession are not adjusted (1.1.12). PGDs are not used for managing long-term conditions, or when uncertainty remains about the differential diagnosis (1.1.14).

The Medical Director's decision of 15 October 2026 to proceed with this PGD, and his reasons against each of these recommendations, are recorded in Why this is a PGD (How to use this PGD): an initiation course only, with mandatory prescriber handover; structured screens, a sleep diary, a Summary Care Record check and referral rules for the differential diagnosis; Yellow Card reporting of every suspected reaction; and an audit he reviews.

What to tell the patient

Daridorexant blocks orexin, a brain chemical that keeps you awake; it is not a benzodiazepine or a "Z-drug". It is taken every night, within 30 minutes of going to bed, not as and when.

It works alongside, not instead of, good sleep habits and CBT-I. The improvement in trials was modest on average: at 3 months on 50 mg, falling asleep about 12 minutes sooner and about 18 minutes less time awake in the night than with a dummy tablet, measured in a sleep laboratory, with smaller differences in what patients themselves noticed.

Do not drive the morning after the first dose; take the first 4 doses only before days with no driving; after that allow at least 9 hours between a dose and driving, and drive only when fully alert. Avoid alcohol in the evening, and grapefruit in the evening.

Report sleep paralysis, vivid frightening hallucinations on falling asleep or waking, sudden muscle weakness with laughter or strong emotion, sleepwalking or doing things in your sleep, and any low mood or thoughts of self-harm.

It is reviewed within 3 months and stopped if it has not helped enough. Any continuation is for a prescriber, who may decide not to continue it.

The Patient Group Direction

How to use this PGD

Why this is a PGD: the Medical Director's decision under NICE MPG2

- On 15 October 2026 the Medical Director of Get Real Health considered two clinical reviews of this PGD and decided to proceed with it as a PGD, limited to an initiation course of three supplies followed by a prescriber. This section records his reasons against NICE MPG2 1.1.1 (a PGD only where it offers an advantage for patient care without compromising safety), 1.1.8 (a black triangle medicine only when clearly justified by best clinical practice) and 1.1.14 (not for managing long-term conditions, or where uncertainty remains about the differential diagnosis). It is a judgement for this service, open to review, not a claim that a PGD is better than prescribing in general.
- ADVANTAGE FOR PATIENT CARE (MPG2 1.1.1). NICE TA922 recommends daridorexant for adults whose long-term insomnia considerably affects daytime functioning when CBT-I has not worked, is not available or is unsuitable. Where NHS prescribing of daridorexant is limited locally and NHS CBT-I is not available, those adults have few routes to a licensed, NICE-recommended treatment. Clinical experts consulted by SMC (SMC2611) noted that benzodiazepines, Z-drugs and melatonin are used off-label for prolonged periods in insomnia. This PGD gives access to the licensed, NICE-recommended option, time-limited, with the 3-month effectiveness review built in. It also brings a structured, recorded assessment to every patient (sleep history, a 1 to 2 week sleep diary, ISI, PHQ-9, GAD-2 or GAD-7, Epworth, STOP-Bang, measured BMI and neck, a Summary Care Record check and an interaction check), which in the Medical Director's judgement is more consistent than an unstructured private consultation, and it delivers brief CBT-I components where no NHS CBT-I is available.
- BLACK TRIANGLE (MPG2 1.1.8). The PGD follows NICE TA922 and SMC2611 exactly and uses only the licensed indication, doses and tablets. The black triangle is shown on the cover, the medicine name and the record. Every suspected adverse reaction, however minor, is reported on a Yellow Card, and the stop rules are set one per symptom, stricter than the SmPC requires.
- INITIATION, NOT LONG-TERM MANAGEMENT (MPG2 1.1.14). The PGD authorises one course of three numbered supplies, never reset, ending with the effectiveness assessment that TA922 and the SmPC require within 3 months. Any continuation, and the periodic reassessment TA922 asks for, is for a prescriber. The patient is told at supply 1 that the GP may decline to continue it.
- DIFFERENTIAL DIAGNOSIS (MPG2 1.1.14). Uncertainty is addressed by structured screens with fixed referral thresholds (STOP-Bang with an alternative high-risk rule, Epworth, PHQ-9, GAD-2 and GAD-7), a 1 to 2 week sleep diary before supply 1, the Summary Care Record check, and referral rules for every other sleep disorder and untreated cause in the exclusion criteria. Where any of these leaves doubt, the patient is referred and not supplied.
- AUDIT. Each adopting pharmacy keeps, and gives Get Real Health on request, an anonymised record of every course: supplies made, ISI at baseline and at each visit, the change in ISI, PHQ-9 and GAD scores, stops by reason (stop rule, early stop at visit 2, inadequate response, gap, interaction), referrals made and whether the prescriber continued treatment, and Yellow Card reports. The Medical Director reviews these results before the review date, and at any time a safety concern arises, and decides whether the PGD continues, changes or is withdrawn.

A black triangle medicine, on-label, for an initiation course only

- Daridorexant is a dual orexin receptor antagonist. It is not a benzodiazepine or a Z-drug and is not a controlled drug in the UK. It is taken every night within 30 minutes before bed, not as and when needed.
- A patient who is helped and wants to continue is handed over to a prescriber at supply visit 3, while they still have a month of tablets. The GP is asked, not instructed, and may decline; a private prescriber is the alternative.

Who this is for

- Adults aged 18 to 75 at the first supply with long-term insomnia: difficulty getting to sleep, staying asleep or waking too early, on 3 or more nights a week for at least 3 months, despite adequate opportunity to sleep, with a considerable effect on daytime functioning, an Insomnia Severity Index (ISI) of 15 or more, and a sleep diary of 1 to 2 weeks that fits that history.
- CBT-I FIRST. NICE TA922 recommends daridorexant only where CBT-I has been tried and has not worked, or is not available, or is unsuitable. "Not available" is judged by NHS access, not by willingness to pay. Free NHS digital CBT-I counts as available wherever it is offered to the patient. Where no NHS CBT-I is available, the pharmacist gives brief CBT-I components at supply 1 and reinforces them at visits 2 and 3 (Appendix 3).

- LOOK FOR ANOTHER CAUSE. Sleep apnoea, excessive daytime sleepiness, restless legs, narcolepsy, circadian rhythm disorders and shift work, depression and anxiety, pain, menopausal symptoms, alcohol, caffeine and medicines that disturb sleep. The screens in Appendix 2 are done before supply 1; PHQ-9 and GAD-2 are repeated at every supply visit.

The numbers

- DOSE: 50 mg once nightly within 30 minutes before bed. 25 mg once nightly ONLY for a patient who regularly takes a moderate CYP3A4 inhibitor (Appendix 4). No other dose. Never more than one tablet a night.
- THREE SUPPLIES, NUMBERED, 30 TABLETS EACH, 25 TO 35 DAYS APART. Each supply visit is also no more than 7 days after the patient's last tablet: more than 7 nights without tablets ends the course, and the patient is referred to a prescriber. Nights withheld on this PGD's instruction because of an interacting medicine are recorded and are not counted as nights without tablets.
- SUPPLY VISIT 2 is an early safety review; the patient may stop there if the ISI is unchanged or worse and they wish to; otherwise supply 2 is made when the criteria are met, including a worse ISI where the patient wishes to continue and no stop rule applies. SUPPLY VISIT 3 is the NICE TA922 effectiveness assessment: an adequate response is ALL of an ISI fall of 6 points or more from baseline, a current ISI of 14 or below, better daytime functioning, and the patient wishing to continue. Anything less: stop, no supply 3.
- Supply visit 3 always falls within 13 weeks of supply 1, which meets the TA922 and SmPC requirement to assess within 3 months.

Driving

- No driving the morning after the first dose. The first 4 doses are taken only on nights before a day with no driving. After that, at least 9 hours between a dose and driving, and only when fully alert (SmPC 4.7). The usual bedtime and morning driving times are recorded and a workable plan is agreed before supply 1. Anyone who must drive or do safety-critical work in the morning and cannot keep 9 hours is not supplied.

Stop rules, one per symptom

- STOP AND SEEK HELP THE SAME DAY: thoughts of suicide or self-harm; a serious allergic reaction (swelling of the face, lips or tongue, or difficulty breathing); driving while not fully awake; or any other activity while not fully awake that caused an injury or could have done. No further supply under this PGD.
- STOP AND CONTACT THE PHARMACY OR GP WITHIN A WEEK: sleep paralysis (being unable to move or speak for a while on falling asleep or waking), seeing or hearing things that are not there on falling asleep or waking, sudden weakness of the legs, face or neck brought on by laughter or strong emotion, sleepwalking or any other activity while not fully awake that you do not remember (for example eating or cooking) where no one was hurt or put at risk, new or worsening low mood or anxiety, still feeling sleepy or slowed in the mornings after the first week, a fall, or a rash. No further supply under this PGD; the patient is referred to the GP. None of these is managed by reducing the dose.
- Headache, nausea, dizziness, tiredness, nightmares or vivid dreams are not stop symptoms on their own. The patient may stop at any time for any reason, without tapering.

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Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

Patient Group Direction

for the supply, for the treatment of Long-Term Insomnia, of:

Daridorexant 50 mg and 25 mg film-coated tablets (QUVIVIQ), black triangle

Patient Group Direction for the supply of daridorexant 50 mg or 25 mg film-coated tablets (QUVIVIQ, black triangle) for long-term insomnia in adults aged 18 to 75 years, as an initiation course of up to three supplies of 30 tablets.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC. This PGD is not used by pharmacy technicians.
- All users must be competent in assessing sleep problems: taking a sleep history, reviewing a sleep diary, distinguishing long-term insomnia from short-term insomnia, and recognising the features of other sleep disorders (obstructive sleep apnoea, narcolepsy, restless legs syndrome, circadian rhythm disorders, parasomnias) that need referral.
- All users must be able to give sleep hygiene advice, deliver the brief CBT-I components in Appendix 3 (stimulus control, a sleep window plan and the sleep diary), signpost to NHS CBT-I services including free NHS digital CBT-I where offered, and apply the definitions of tried, not available and unsuitable in this PGD.
- All users must be competent in administering and scoring the Insomnia Severity Index, PHQ-9, GAD-2 and GAD-7, the Epworth Sleepiness Scale and the STOP-Bang questionnaire as set out in Appendix 2, using the licensed versions held by the pharmacy, in measuring height, weight and neck circumference, and in responding to a positive answer about suicide or self-harm, including same-day and emergency referral routes.
- All users must be competent in checking for CYP3A4 and P-gp interactions and CNS depressants using the SmPC, the BNF interactions checker and Appendix 4, including medicines bought over the counter and herbal products, and in using the Summary Care Record with the patient's consent.
- All users must know the warnings in the SmPC (next-day impairment and driving, sleep paralysis, hallucinations, cataplexy-like symptoms, depression and suicidal ideation, abuse potential, falls in the elderly) and the stop rules in this PGD, and must understand the reporting requirements for a black triangle medicine.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required. All users must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005.

The PGD

Indication	Long-term insomnia in an adult aged 18 to 75 at the first supply: insomnia symptoms on 3 or more nights a week for at least 3 months, with a

	considerable effect on daytime functioning, where CBT-I has been tried and has not worked, or is not available, or is unsuitable (NICE TA922; SMC2611). This is the licensed indication (SmPC 4.1) within the NICE positioning. QUVIVIQ is a black triangle medicine. Supply under this PGD is a private service, face to face on the pharmacy premises, as a single initiation course of up to three supplies, on the Medical Director's decision recorded in the opening pages.
Inclusion criteria	**At the first supply, ALL of the following.** ● **Aged 18 to 75 years inclusive. A patient who turns 76 during the course may complete it.** ● **Difficulty getting to sleep, staying asleep or waking too early, or unrefreshing sleep, on 3 or more nights a week for at least 3 months, despite adequate opportunity and circumstances for sleep, by the patient's account. Record the pattern, the duration and the usual times of going to bed and getting up.** ● **SLEEP DIARY: a sleep diary kept for at least 7 and up to 14 consecutive nights before supply 1 (Appendix 2), consistent with the history. A patient without one is given a diary and supply 1 is booked for when it is complete.** ● **A considerable effect on daytime functioning, described by the patient in their own words (for example poor concentration or memory, mistakes at work, low mood or irritability, fatigue), and recorded; AND an Insomnia Severity Index (ISI) score of 15 or more at supply 1, on the pharmacy's licensed version (Appendix 2), recorded as the baseline.** ● **Sleep hygiene advice (Appendix 3) has been followed for at least the last 4 weeks, by the patient's account, without adequate improvement, and is given again in writing.** ● **CBT-I, ONE of the following, recorded with the reason:** ○ TRIED AND NOT WORKED: the patient has completed, or engaged with for at least 4 weeks, a structured CBT-I programme (face to face, group, or digital such as Sleepio or SilverCloud Space for Sleep), within the last 2 years, without adequate improvement. Record the programme, the approximate dates and how far the patient got. The patient's account is sufficient; written evidence is recorded where the patient has it. ○ NOT AVAILABLE: judged by NHS access, not by willingness to pay. No NHS CBT-I (face to face, group or digital) is offered to the patient where they live, having checked the local NHS Talking Therapies offer, any locally commissioned digital CBT-I and the national offer (Appendix 3); or the only NHS option is face to face with a waiting time of more than 3 months and no NHS digital option is offered. Free NHS digital CBT-I counts as AVAILABLE wherever it is offered to the patient (for example Sleepio for people living in Scotland, SilverCloud Space for Sleep for people living in Wales, and Sleepio in England where locally commissioned). Record what was checked. Tell the patient that paid private and digital programmes exist, and record their decision; declining to pay does not affect eligibility, and neither does a decision to pay. ○ UNSUITABLE: CBT-I cannot reasonably be delivered to this patient, for example because of a cognitive, sensory or language barrier for which no adapted programme is available, no access to or ability to use a digital programme where no face to face option exists, or a reason the patient's GP or a sleep or mental health clinician has recorded. Record the specific reason. A preference not to try CBT-I is not

unsuitability: that patient is signposted and not supplied.

- **Where CBT-I is NOT AVAILABLE or UNSUITABLE, the pharmacist delivers the brief CBT-I components in Appendix 3 at supply 1 (stimulus control, a sleep window plan, continuing the sleep diary) and reinforces them at supply visits 2 and 3.**
- **STOP-Bang below 5, AND not 2 or more of the four STOP items together with male sex, BMI over 35 or neck circumference over 40 cm, AND no witnessed pauses in breathing, choking or gasping in sleep (Appendix 2). Height, weight and neck measured in the pharmacy. STOP-Bang is not used for a patient with diagnosed OSA using CPAP most nights; that patient is assessed under Cautions, and this criterion (including the witnessed pauses question) does not apply to them.**
- **Epworth Sleepiness Scale 10 or below (Appendix 2).**
- **PHQ-9 total below 10, and question 9 (thoughts of being better off dead or of self-harm) answered "not at all" (Appendix 2).**
- **GAD-2 below 3; or, where GAD-2 is 3 or more, GAD-7 below 10 (Appendix 2).**
- **DRIVING PLAN: the usual bedtime, the usual earliest time of driving or other activity needing full alertness on each day of the week, and whether the patient drives for work are recorded. A workable plan is agreed before supply: no driving the morning after the first dose; the first 4 doses taken only on nights before a day with no driving; after that at least 9 hours between a dose and driving, and driving only when fully alert; and the dose skipped on any night that is not possible.**
- **MEDICINES AND ALLERGIES: medicines checked against Appendix 4, including over the counter, herbal and recreational products, and allergies recorded. SUMMARY CARE RECORD: where the patient has a Summary Care Record, or an equivalent national record the pharmacy can access, it is checked with the patient's consent; a patient who refuses consent to the check is not supplied. Where no record exists or it cannot be accessed, record why. No excluded medicine. Over the counter and other sleep aids stopped as set out under Cautions.**
- **Registered with a GP and agrees to the GP being informed of the supply, of any stop for a side effect and of the outcome of the effectiveness assessment. A patient who will not allow the GP to be told is not supplied.**
- **Understands and accepts that this PGD covers at most three supplies, that treatment stops if it has not helped enough at supply visit 3, that any continuation is a prescriber's decision, that the GP may decline to continue it, and that a private prescriber is the alternative. Record that this was explained.**
- Valid informed consent, and understands that this is a private service and what it costs.
- No exclusion criterion present.

At supply visit 2 (supply 2): one supply under this PGD so far; the visit is 25 to 35 days after supply 1 (earlier only under the Cautions rule for a regular moderate CYP3A4 inhibitor started during the course; later only as set out under Dose and frequency for recorded withheld nights) and no more than 7 days after the last tablet; no stop symptom since supply 1; no new excluded medicine or condition, with the Summary Care Record rechecked where available; PHQ-9 total below 10 with question 9 "not at all"; GAD-2 below 3 (or GAD-7 below 10); nights taken out of the last 14 recorded; ISI recorded; and the patient wishes to continue. If the ISI is unchanged or higher than baseline and the patient wishes to stop, the course stops here (early stop). Otherwise supply 2 is made when these criteria are met, including where the ISI is unchanged or worse but the patient wishes to continue and no stop rule applies.

	At supply visit 3 (supply 3, the last): two supplies under this PGD so far; the visit is 25 to 35 days after supply 2 (extended only as set out under Dose and frequency for recorded withheld nights) and no more than 7 days after the last tablet; no stop symptom since supply 2; no new excluded medicine or condition, with the Summary Care Record rechecked where available; PHQ-9 total below 10 with question 9 "not at all"; GAD-2 below 3 (or GAD-7 below 10); tablets taken on at least 10 of the last 14 nights on which a dose was due (nights withheld on this PGD's instruction are not due, and the count goes back past them); AND an adequate response as defined under Dose and frequency.
Exclusion criteria	**Refer, do not supply, where any of the following applies.** ● Under 18, or aged 76 or over at the first supply. ● Hypersensitivity to daridorexant or to any excipient (SmPC section 6.1: mannitol, microcrystalline cellulose, povidone, croscarmellose sodium, silicon dioxide, magnesium stearate, hypromellose, glycerol, talc, titanium dioxide, iron oxides). ● **NARCOLEPSY (contraindicated), or features suggesting it or another cause of excessive sleepiness: an Epworth score above 10; falling asleep unintentionally during the day (while talking, eating, reading or watching television, as a passenger, or at any time while driving); sudden muscle weakness brought on by laughter or emotion (cataplexy); or recurrent sleep paralysis or hallucinations on falling asleep or waking before any treatment.** ● **Possible SLEEP APNOEA, in a patient without diagnosed OSA using CPAP most nights: STOP-Bang 5 or more; OR 2 or more of the four STOP items together with male sex, BMI over 35 or neck circumference over 40 cm; OR witnessed pauses in breathing, choking or gasping in sleep at any score. Diagnosed sleep apnoea not being treated, or where the patient is not using their CPAP most nights. Refer to the GP for assessment.** ● **Another sleep disorder: restless legs (an urge to move the legs in the evening or at rest, relieved by moving), a circadian rhythm disorder (consistently unable to fall asleep until very late, or very early waking with early sleepiness, with normal sleep when allowed to keep those hours), night or rotating shift work that includes nights, or a history of sleepwalking, sleep-eating, sleep-driving or other complex behaviour in sleep.** ● Insomnia that appears to be mainly caused by something untreated or not yet assessed: uncontrolled pain; breathlessness lying flat or at night; frequent night-time urination not yet investigated; menopausal hot flushes and night sweats not yet discussed with a clinician; symptoms of an overactive thyroid; a medicine started or changed in the 3 months before the insomnia began (for example a stimulant, corticosteroid, decongestant, beta-blocker or SSRI); or caffeine after mid-afternoon or energy drinks that the patient has not tried stopping. ● **DEPRESSION, ANXIETY AND MENTAL HEALTH: PHQ-9 total of 10 or more; GAD-7 of 10 or more; ANY answer other than "not at all" to PHQ-9 question 9, or any current thoughts of suicide or self-harm, or self-harm or a suicide attempt in the last 2 years (same-day assessment, see Actions); diagnosed bipolar disorder, psychosis or schizophrenia, or an eating disorder; post-traumatic stress disorder under active treatment, or with nightmares; current care under a secondary care mental health team; or depression or anxiety whose treatment has started or changed in the last 3 months.** ● **SUBSTANCES: current or past misuse of, or dependence on, alcohol, prescribed medicines or recreational drugs; drinking more than 14 units of alcohol a week; regular use of cannabis or cannabis-based products. The SmPC advises careful follow-up of anyone with a history of abuse or addiction, which a three-supply**

	PGD cannot provide. ● **OTHER HYPNOTICS AND SEDATIVES taken regularly and continuing (Appendix 4): a benzodiazepine, a Z-drug (zopiclone, zolpidem), melatonin from any source (prescribed, bought or imported), a sedating antidepressant (mirtazapine, trazodone, any tricyclic including low-dose amitriptyline), an antipsychotic, gabapentin or pregabalin, an opioid including codeine or tramadol, an antiepileptic, a sedating antihistamine taken most nights, or a muscle relaxant such as baclofen or tizanidine. The SmPC says dose adjustment of either medicine may be needed, which a PGD cannot provide. A prescribed medicine is never stopped to allow supply: refer. A sleep aid that was not prescribed (a bought sedating antihistamine, melatonin bought or imported, a herbal sedative) and that the patient agrees to stop is handled under the single sleep aid rule in Cautions instead.** ● **STRONG CYP3A4 INHIBITORS (contraindicated, Appendix 4), for example itraconazole, ketoconazole, posaconazole, voriconazole, clarithromycin, ritonavir or cobicistat in any combination including nirmatrelvir with ritonavir (Paxlovid), and any medicine its SmPC or the BNF describes as a strong CYP3A4 inhibitor.** ● **CYP3A4 INDUCERS (moderate or strong, Appendix 4), for example carbamazepine, phenytoin, phenobarbital, rifampicin, rifabutin, efavirenz, enzalutamide, apalutamide, modafinil, bosentan or St John's wort, and any medicine its SmPC or the BNF describes as a moderate or strong CYP3A4 inducer. Exposure is substantially reduced and the treatment may not work.** ● Narrow therapeutic index medicines affected by daridorexant 50 mg (Appendix 4): tacrolimus, sirolimus, everolimus, ciclosporin, digoxin, dabigatran, colchicine taken regularly, or simvastatin 80 mg. ● **LIVER: any known chronic liver disease, including cirrhosis or hepatic impairment of any degree: refer. The SmPC requires 25 mg in moderate impairment and advises against use in severe impairment; the degree cannot be established in the pharmacy.** ● **RESPIRATORY: severe COPD (FEV1 below 40 per cent of predicted where known, home oxygen, or a hospital admission for COPD in the last 12 months), chronic respiratory failure, obesity hypoventilation, or a neuromuscular condition affecting breathing. Asthma and mild or moderate COPD are not excluded.** ● **Pregnancy, planning a pregnancy during the course, or breastfeeding.** ● **NIGHT-TIME CARING: the sole night-time carer of an infant or of a dependent adult who may need them during the night.** ● **Aged 65 or over with a fall in the last 12 months, using a walking aid, or getting up at night to the toilet 2 or more times on most nights; or dementia or any condition affecting memory.** ● **DRIVING AND SAFETY-CRITICAL WORK: anyone who must drive, or do other work needing full alertness, early in the morning and cannot reliably keep at least 9 hours between taking a dose and starting, whatever their licence; anyone who cannot arrange 4 nights before days with no driving for the first 4 doses; a Group 2 (lorry or bus) licence holder who drives for work; or a safety-critical role with its own fitness rules (for example pilot, air traffic controller, train driver, or on-call work needing full alertness within 9 hours of bedtime). Refer to the GP or occupational health.** ● **Already taking daridorexant or another orexin receptor antagonist on prescription or from any other source; a course under this PGD already started or completed at any pharmacy,**

	whatever its outcome (by the patient's account and pharmacy records; record the answer); or more than 7 nights without tablets, or a visit outside the supply interval, during the course. ● **Refuses consent to the Summary Care Record check where a record exists.** ● **At supply visits 2 and 3: any stop symptom since the last supply (see Stop rules under Dose and frequency).** ● **At supply visit 3: an effectiveness assessment that does not show an adequate response, or tablets taken on fewer than 10 of the last 14 nights on which a dose was due, so that the response cannot be judged.** ● Not registered with a GP, will not allow the GP to be informed, or lacks capacity to consent.
Cautions	**NEXT-DAY IMPAIRMENT AND DRIVING (SmPC 4.7). In a simulator study driving was impaired the morning after the first 50 mg dose, 9 hours after taking it, but not after 4 consecutive nights. So: no driving the morning after the first dose; the first 4 doses only on nights before a day with no driving; after that at least 9 hours between a dose and driving, cycling or operating machinery, and only when fully alert; and skip the dose on any night when 9 hours is not possible. It is an offence to drive while unfit through drugs, including prescribed medicines.** Aged 65 to 75. The SmPC advises caution in the elderly because of the general risk of falls, although trials did not show more falls than placebo. Increased body sway on being woken 4 hours after a dose was measured in healthy adults under 65 (SmPC 5.1). Advise a night light, a clear path to the toilet, and getting up slowly. Stable, treated depression or anxiety with PHQ-9 below 10 and GAD-2 below 3 (or GAD-7 below 10 where done), on unchanged treatment for at least 3 months (for example an SSRI; citalopram had no relevant interaction in the SmPC). The SmPC advises caution and data are limited. Repeat PHQ-9 and GAD-2 at every supply visit and apply the exclusion if a threshold is reached. Diagnosed obstructive sleep apnoea treated with CPAP used most nights, or mild to moderate COPD. STOP-Bang is not used for a patient with diagnosed OSA using CPAP most nights; that patient is assessed here: confirm and record that CPAP is used most nights (a patient not using it most nights is excluded). The SmPC found no worsening of apnoea or oxygen saturation; advise continuing CPAP. **MODERATE CYP3A4 INHIBITOR TAKEN REGULARLY at the first supply (Appendix 4, for example diltiazem or verapamil), at a stable dose and expected to continue for the rest of the course: the dose is 25 mg. A patient who stops the inhibitor finishes the 25 mg tablets in hand; the next supply is 50 mg.** **MODERATE CYP3A4 INHIBITOR STARTED REGULARLY DURING THE COURSE: the patient stops daridorexant from the first dose of the inhibitor and returns to the pharmacy. The patient returns within 7 days of stopping; if not, the course ends and the patient is referred to the GP. The 50 mg tablets in hand are not taken (MPG2 1.1.12: no dose change to a medicine in the patient's possession) and are returned for destruction. The nights stopped are recorded as withheld nights. A further supply is made only at 25 mg, only if every criterion for that supply visit is met, and counts as the next numbered supply: if it is supply 2, the 25-day minimum does not apply; if it is supply 3, it needs the effectiveness assessment and at least 25 days since supply 2. Otherwise refer to the GP.** **A SHORT COURSE of a moderate or strong CYP3A4 inhibitor started during the course (for example erythromycin, ciprofloxacin, fluconazole, clarithromycin, Paxlovid): do not take daridorexant from the first dose of that medicine until 7 days after the last dose, then resume. The patient tells the pharmacy when this happens, and the withheld nights are**

	recorded. The dose in the patient's possession is never changed. Other CYP3A4 or P-gp substrates not excluded (for example simvastatin up to 40 mg, atorvastatin): no dose change is needed; advise the patient to report unexplained muscle pain. Hormonal contraceptives, warfarin, rosuvastatin and acid-reducing medicines are not affected. Alcohol has additive effects on alertness. Advise no alcohol on the evening of a dose. Grapefruit and grapefruit juice in the evening raise exposure and are avoided. **OVER THE COUNTER AND OTHER SLEEP AIDS (the single rule): sedating antihistamines sold for sleep (for example diphenhydramine or promethazine), melatonin from any source and herbal sedatives such as valerian are stopped before the course and not used during it. The first dose of daridorexant is taken at least 48 hours after the last dose of a long-acting sedating antihistamine such as promethazine, which allows for its long elimination half-life; for other sleep aids, at least the night before.** An occasional single dose of another sedating medicine taken for a different reason (for example chlorphenamine for hay fever or a codeine-containing painkiller): do not take daridorexant that night. Abuse potential. Doses of 100 mg and 150 mg produced drug liking similar to zolpidem in recreational drug users. Supply no more than this PGD allows, at the intervals it allows, and treat a request for an early supply or a report of lost tablets as a reason to refer, not to supply. Women of childbearing potential: no contraception requirement. Advise stopping and telling the GP if pregnancy occurs.
Actions if the patient is excluded or declines	● Explain the reason, and give sleep hygiene advice and the CBT-I signposting in Appendix 3 in writing. ● **NHS CBT-I available and suitable but not yet tried, including a patient who would prefer not to try it: signpost to it (NHS Talking Therapies, free NHS digital CBT-I where offered, or the GP for local options), and invite the patient to return if it has not worked after at least 4 weeks of engagement.** ● **SUICIDE OR SELF-HARM RISK (any answer other than "not at all" to PHQ-9 question 9, disclosed thoughts, or self-harm or a suicide attempt in the last 2 years): do not leave it there. Ask about plans and intent. Immediate danger or a plan: call 999 or arrange accompanied attendance at A&E. Otherwise: same-day contact with the GP, or NHS 111 (option 2 for mental health in England), and give the Samaritans number (116 123). Record what was done.** ● PHQ-9 of 10 or more, GAD-7 of 10 or more without risk, or any other mental health exclusion: refer to the GP routinely, or advise self-referral to NHS Talking Therapies. ● **Possible sleep apnoea (by either STOP-Bang rule or witnessed apnoea), an Epworth score above 10, narcolepsy, restless legs or another sleep disorder: refer to the GP, explaining why. A patient who falls asleep unintentionally while driving, or with a high Epworth score who drives, is advised not to drive until assessed, and that DVLA rules may apply.** ● Interacting medicine, liver, respiratory, falls, driving or occupational, night-time caring or substance exclusion: refer to the GP, who may prescribe with dose adjustment or monitoring a PGD cannot provide. ● Document the reason, the advice given and the referral made. Inform the GP where a referral is made.
Yellow Card reporting	QUVIVIQ is a black triangle medicine. Report EVERY suspected adverse reaction, however minor and whether or not it is listed in the SmPC, via https://yellowcard.mhra.gov.uk, and record the report in the patient record and

	in the audit dataset. Inform the GP of any reaction that leads to stopping.
Records to be kept	● **Date and time; patient name, address, date of birth, and the GP with whom they are registered; ALLERGIES and previous adverse reactions; consent; and that the GP was informed within 7 days of supply 1, within 7 days of any stop for a side effect, and at supply visit 3 of the outcome of the effectiveness assessment.** ● **The sleep history: pattern, nights per week, duration in months, usual bed and rising times, the daytime effects in the patient's words, the factors considered and excluded as causes, and a summary of the sleep diary (nights covered, usual time to fall asleep, time awake in the night, total sleep).** ● **Screening at supply 1: ISI (baseline), PHQ-9 total and question 9 answer, GAD-2 (and GAD-7 where done), Epworth score, STOP-Bang score with the number of STOP items, measured height, weight, BMI and neck circumference, and the witnessed apnoea answer. At supply visits 2 and 3: ISI, PHQ-9 with question 9, GAD-2 (and GAD-7 where done).** ● Sleep hygiene: that it had been followed for at least 4 weeks and that written advice was given again. ● **CBT-I: which of tried and not worked, not available, or unsuitable applies, with the programme and dates, the NHS offers checked, or the specific reason; the patient's decision about paid private or digital CBT-I; and, where given, the brief CBT-I components delivered at each visit.** ● **Medicines checked, including over the counter, herbal and recreational products; the Summary Care Record check with consent (or why it was not possible); when any sleep aid was stopped; and any moderate CYP3A4 inhibitor that sets the dose at 25 mg.** ● **The driving plan: usual bedtime, morning driving times, whether driving is for work, and the agreed plan for the first 4 doses and the 9-hour rule.** ● **The product supplied (QUVIVIQ 50 mg or 25 mg, black triangle) with its marketing authorisation number from the pack, the dose, 30 tablets, the batch number and expiry date, and the SUPPLY NUMBER (1, 2 or 3) with the date of supply 1.** ● **The words "supply under Get Real Health PGD Long-Term Insomnia (Daridorexant), version 001".** ● **At supply visits 2 and 3: the date of the last tablet, the nights taken out of the last 14, any nights withheld on this PGD's instruction because of an interacting medicine (the medicine and the dates), and any early stop at visit 2.** ● **At supply visit 3: the baseline and current ISI, the change, whether daytime functioning has improved by the patient's account, whether the patient wishes to continue, the decision (adequate response, supply 3 and handover letter; or not adequate, stopped), and the date the letter was sent.** ● **That the driving advice, the stop rules, the 3-supply limit, and the possibility that the GP may decline to continue treatment were explained and given in writing.** ● **Any adverse reaction, the action taken, and the Yellow Card report.** ● Name and GPhC registration number of the pharmacist. ● Advice given, including advice given if the patient is excluded or declines. Records signed, dated, legible and contemporaneous, kept for 8 years. The patient is given a written record of the dose, the stop rules, the driving plan and the date window for their next supply visit. The anonymised audit dataset

	described in the opening pages is kept and given to Get Real Health on request.

The medicine

Name, form and strength	● **QUVIVIQ 50 mg film-coated tablets (daridorexant hydrochloride equivalent to 50 mg daridorexant), Idorsia, PLGB 48711/0003: light orange arc-triangle shaped film-coated tablets, debossed "50" on one side and "i" on the other.** ● **QUVIVIQ 25 mg film-coated tablets (daridorexant hydrochloride equivalent to 25 mg daridorexant), Idorsia, PLGB 48711/0002: light purple arc-triangle shaped film-coated tablets, debossed "25" on one side and "i" on the other. For a patient who regularly takes a moderate CYP3A4 inhibitor only.** ● **BLACK TRIANGLE: subject to additional monitoring. Record the marketing authorisation number printed on the pack supplied.** ● Excipients: mannitol, microcrystalline cellulose, povidone, croscarmellose sodium, silicon dioxide, magnesium stearate; film coat hypromellose, microcrystalline cellulose, glycerol, talc, titanium dioxide, iron oxides. No lactose. Less than 1 mmol sodium per tablet.
Legal category	POM. Not a controlled drug in the UK.
Dose and frequency	● **50 mg (one tablet) once nightly, taken by mouth in the evening within 30 minutes before going to bed. Licensed dose (SmPC 4.2).** ● **25 mg (one tablet) once nightly ONLY where the patient regularly takes a moderate CYP3A4 inhibitor (Appendix 4) at a stable dose and expects to continue it for the rest of the course (SmPC 4.2 and 4.5). No other reason for 25 mg applies under this PGD. Each supply is at the dose that applies on the day of that supply; the dose of tablets already supplied is never changed.** ● Maximum one tablet in 24 hours. Taken every night, not as and when needed. A dose forgotten at bedtime is not taken during the night. Skip the dose on any night when there will not be at least 9 hours before driving or another activity needing full alertness. May be taken with or without food, but not soon after a large meal. ● **SUPPLY 1: 30 tablets, once the inclusion criteria, including the sleep diary and the driving plan, are met. The brief CBT-I components (Appendix 3) are delivered where CBT-I is not available or unsuitable.** ● **TIMING OF SUPPLY VISITS 2 AND 3: 25 to 35 days after the previous supply, and no more than 7 days after the patient's last tablet; the patient is asked to attend in the last few days before running out. More than 7 nights without tablets, for any reason other than nights withheld on this PGD's instruction (an interacting short course, or waiting to return after starting a regular moderate CYP3A4 inhibitor), ends the course: refer to a prescriber. Where recorded withheld nights mean the tablets last longer, the 35-day limit is extended by the number of withheld nights, provided supply visit 3 is still within 13 weeks of supply 1; otherwise refer.** ● **SUPPLY VISIT 2: early safety review (stop symptoms, next-day effects and driving, mood with PHQ-9 and GAD-2, nights taken out of the last 14, new medicines and the Summary Care Record, brief CBT-I reinforced). ISI recorded. EARLY STOP: if the ISI is unchanged or higher than baseline and the patient wishes to stop, no supply 2; the GP is informed. Otherwise SUPPLY 2 (30 tablets) is made when the supply visit 2 criteria are met, including where the ISI is unchanged or worse but the patient**

	wishes to continue and no stop rule applies. ● **SUPPLY VISIT 3: the NICE TA922 EFFECTIVENESS ASSESSMENT. First record the nights taken out of the last 14 on which a dose was due (nights withheld on this PGD's instruction are not due, and the count goes back past them); at least 10 are needed for the response to be judged; with fewer, no supply 3 and refer to the GP. ADEQUATE RESPONSE means ALL of: (a) the ISI has fallen by 6 points or more from the supply 1 score; (b) the current ISI is 14 or below; (c) the patient reports that their daytime functioning is better; and (d) the patient wishes to continue. With an adequate response and the other supply visit 3 criteria met, SUPPLY 3: 30 tablets, and the handover letter is sent the same day. Without an adequate response: STOP, no supply 3; the patient stops the next night (no tapering is needed), is advised to re-engage with CBT-I, and the GP is informed.** ● SOURCE OF THE RESPONSE THRESHOLD: the Medical Director's decision, informed by SMC2611 Table 2.2, in which the mean fall in ISI at 3 months was 7.2 points on daridorexant 50 mg and 5.4 points on placebo. A fall of 6 alone sits between those means, which is why it is combined with a current ISI of 14 or below (below the moderate range of 15 to 21), better daytime functioning and the patient's wish to continue. It is not a responder definition published by NICE or SMC. ● **MAXIMUM DURATION: three supplies of 30 tablets (90 tablets, about 13 weeks of nightly treatment) under this PGD, once per patient, counted by supply number from supply 1 and never reset by a gap, a stop or a change of pharmacy. There is no fourth supply under this PGD in any circumstance.** ● **HANDOVER LETTER at supply visit 3, to the patient's GP or another prescriber the patient chooses. It is information plus a request: the baseline and current ISI, PHQ-9 and GAD scores, the dose, the supply dates, adherence, any adverse effects, the CBT-I position, that NICE TA922 requires continued treatment to be reassessed at regular intervals, and a request that the prescriber consider whether to continue. It states that the decision is the prescriber's, and that if the GP does not continue, the patient may see a private prescriber. The patient is told the same, and that if treatment does not continue, stopping is safe without tapering.** ● **STOP RULES, one rule per symptom, never managed by reducing the dose. STOP AND SEEK HELP THE SAME DAY for: thoughts of suicide or self-harm; a serious allergic reaction (swelling of the face, lips or tongue, or difficulty breathing); driving while not fully awake; or any other activity while not fully awake that caused an injury or could have done. STOP AND CONTACT THE PHARMACY OR GP WITHIN A WEEK for: sleep paralysis (being unable to move or speak for a while on falling asleep or waking), seeing or hearing things that are not there on falling asleep or waking, sudden weakness of the legs, face or neck brought on by laughter or strong emotion, sleepwalking or any other activity while not fully awake that you do not remember (for example eating or cooking) where no one was hurt or put at risk, new or worsening low mood or anxiety, still feeling sleepy or slowed in the mornings after the first week, a fall, or a rash. After any stop rule: no further supply under this PGD; refer to the GP; Yellow Card.** ● STOPPING for any other reason: may be stopped at any time without tapering. No rebound insomnia was seen after 3 months of treatment in trials (SmPC 5.1).
Quantity to be supplied	30 tablets (one original pack of 30) of the 50 mg or 25 mg strength at each of up to three supplies. Maximum 90 tablets in total under this PGD per patient. Supply as a dispensed medicine, labelled, with the manufacturer's patient

	information leaflet.
Route and method of administration	Oral. Swallow the tablet whole with water, within 30 minutes before going to bed.
Labelling	Label as a dispensed medicine with the patient's name, "Take ONE tablet within 30 minutes before going to bed", the date and the pharmacy, and add: "This medicine makes you sleepy. Do not drive the morning after your first dose. Take your first 4 doses only on nights before a day you will not drive. After that, allow at least 9 hours before driving and only drive if you feel fully alert. Do not drink alcohol." Supply the manufacturer's patient information leaflet.
Storage	No special storage conditions. Keep out of the sight and reach of children.
Adverse effects	From the SmPC. Common: headache, somnolence, dizziness, fatigue, nausea. Uncommon: hypersensitivity including rash and urticaria, hallucinations, abnormal dreams and nightmares, sleepwalking, sleep paralysis. Sleep paralysis and hallucinations occur mainly in the first weeks. Cataplexy-like symptoms have been reported with the class. Worsening of depression and suicidal thoughts have been reported with hypnotics in people with depression. No dose relationship was seen. Overdose (up to 200 mg studied): somnolence, muscle weakness, poor attention, fatigue, headache, constipation; no antidote, supportive care, refer to A&E.

Information for the patient

Written information	Supply the manufacturer's patient information leaflet, and a written record of the dose, the stop rules, the driving plan, the sleep hygiene advice and brief CBT-I plan or CBT-I signposting, and the date window for the next supply visit.
Counselling	• **Take one tablet every night, within 30 minutes before you go to bed. Do not take it if you forget at bedtime and wake in the night. Do not take more than one in a night.** • It helps some people fall asleep sooner and stay asleep longer, but on average the difference is modest. Keep going with your sleep routine, your sleep diary and CBT-I; they work alongside it. • **DRIVING: do not drive the morning after your first tablet. Take your first 4 tablets only on nights before a day when you will not drive. After that, leave at least 9 hours between taking a tablet and driving, cycling or using machines, and only drive if you feel fully alert. If you cannot leave 9 hours, skip that night's tablet. It is against the law to drive if a medicine affects your driving.** • No alcohol on the evenings you take it, and no grapefruit or grapefruit juice in the evening. Do not use other sleep aids, including melatonin and sleeping tablets bought in a shop, during the course. Do not take it on a night when you have taken another medicine that makes you drowsy. • **Before starting any new medicine, including antibiotics, antifungals and herbal remedies such as St John's wort, tell the pharmacist or doctor that you take daridorexant. Some antibiotics and antifungals mean you must not take it for the course and for 7 days after: tell us when this happens so we can record it. If a doctor starts you on a medicine you will take regularly, such as diltiazem or verapamil, stop daridorexant and come back to us within 7 days, before taking any more.** • **STOP AND SEEK HELP THE SAME DAY (your GP, NHS 111, or 999 in an emergency) if you have thoughts of suicide or self-harm; a serious allergic reaction (swelling of the face, lips or tongue, or difficulty breathing); driving while not fully awake; or**

	any other activity while not fully awake that caused an injury or could have done. ● **STOP AND CONTACT US OR YOUR GP WITHIN A WEEK if you have sleep paralysis (being unable to move or speak for a while on falling asleep or waking), seeing or hearing things that are not there on falling asleep or waking, sudden weakness of the legs, face or neck brought on by laughter or strong emotion, sleepwalking or any other activity while not fully awake that you do not remember (for example eating or cooking) where no one was hurt or put at risk, new or worsening low mood or anxiety, still feeling sleepy or slowed in the mornings after the first week, a fall, or a rash.** ● Headache, feeling sick, dizziness, tiredness and vivid dreams can happen and usually settle. You can stop at any time; you do not need to cut down first. ● This is a new medicine being closely monitored. Tell us about any side effect, however small; we report it to the MHRA, and you can too on the Yellow Card website. ● **Come back for your next supply between 25 and 35 days after this one, in the last few days before you run out. If you go more than 7 nights without tablets we cannot continue. At your second visit you can stop if it has not helped. At your third visit we check whether it has helped enough. If it has, you get one more month and we write to your GP or another prescriber with the results and ask them to consider continuing. Your GP may decide not to continue it; if so, you could see a private prescriber. If it has not helped enough, you stop. This service gives at most three supplies.** ● If you become pregnant, stop and tell your GP. Keep the tablets safe and do not share them.
Follow-up	Supply visit 2 at 25 to 35 days after supply 1, and supply visit 3 at 25 to 35 days after supply 2, in person, each no more than 7 days after the last tablet, with ISI, PHQ-9 and GAD-2 at each and brief CBT-I reinforced where it was given. GP informed within 7 days of supply 1 and of any stop for a side effect; handover letter at supply visit 3 where the response is adequate, or a stop notification where it is not or where the course ends early. Same-day referral for the same-day stop symptoms; GP within a week for the other stop symptoms. A patient mid-course when this version expires continues under the successor version, or is referred to a prescriber if there is none.

Key references

- QUVIVIQ 50 mg and 25 mg film-coated tablets, Summary of Product Characteristics, Idorsia, PLGB 48711/0003 and PLGB 48711/0002, text revised 12 May 2025, https://www.medicines.org.uk/emc/product/15360/smpc and https://www.medicines.org.uk/emc/product/15359/smpc
- NICE TA922, Daridorexant for treating long-term insomnia, published 18 October 2023, https://www.nice.org.uk/guidance/ta922
- Scottish Medicines Consortium, SMC2611, daridorexant (Quviviq), advice 8 March 2024, published 8 April 2024.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- QUVIVIQ 25 mg and 50 mg film-coated tablets SmPC (Idorsia, PLGB 48711/0002 and PLGB 48711/0003, text revised 12 May 2025, black triangle), medicines.org.uk; NICE Technology Appraisal TA922, Daridorexant for treating long-term insomnia, published 18 October 2023; Scottish Medicines Consortium SMC2611, daridorexant (Quviviq), advice 8 March 2024, published 8 April 2024; NICE Medicines Practice Guideline MPG2, Patient Group Directions; NICE Clinical Knowledge Summary, Insomnia; NHS Grampian (Sleepio in Scotland) and NHS Wales SilverCloud self-referral guide. Summarised 15 October 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Signed on behalf of Get Real Health

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v001
Supersedes	None (new PGD)
Valid from date	15 October 2026
Expiry date	31 July 2027
Review date	31 May 2027, or earlier if the QUVIVIQ SmPC changes, the black triangle is removed, NICE TA922 is updated, the controlled drug status of daridorexant changes, or the Medical Director's audit review raises a concern

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signature:  Date: 15 October 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signature:  Date: 15 October 2026

Both authorising signatories reviewed this version together on 15 October 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: .....

	Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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Valid from date	15 October 2026
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Change history

Version	v001
Date	15 October 2026
Changes	● New PGD. Daridorexant (QUVIVIQ 50 mg and 25 mg, Idorsia) for long-term insomnia, ordered by the Medical Director for a pharmacy group's sleep service. Written by Get Real Health from the GB SmPCs, NICE TA922 (published 18 October 2023), SMC2611 and NICE MPG2, independently of the manufacturer. It replaces nothing. ● Revised before issue on 15 October 2026 after two clinical reviews. The Medical Director decided to proceed as a PGD, as an initiation course of three supplies followed by a prescriber. His reasons are recorded in a new opening section against NICE MPG2 1.1.1 (advantage for patient care: access where NHS prescribing is limited and NHS CBT-I is unavailable, a licensed alternative to the prolonged off-label hypnotic use SMC2611 describes, and a structured recorded assessment), 1.1.8 (black triangle) and 1.1.14 (initiation only with mandatory prescriber handover; differential diagnosis addressed by structured screens, sleep diary, Summary Care Record check and referral rules), with an audit of outcomes, ISI change, stop rates, Yellow Cards and prescriber acceptance reviewed by the Medical Director. ● Driving (SmPC 4.7): no driving the morning after the first dose; the first 4 doses only on nights before a day with no driving; afterwards at least 9 hours and driving only when fully alert; bedtime and morning driving times recorded and a workable plan agreed before supply 1. Anyone who must drive or do safety-critical work early in the morning and cannot keep 9 hours is excluded, not only Group 2 drivers. The same wording is in the counselling and on the label. ● Interactions: a regular moderate CYP3A4 inhibitor started during the course means stopping daridorexant and returning; further supply only at 25 mg if every criterion is met, otherwise referral. The 25 mg rule now requires a stable dose expected to continue for the rest of the course, in place of a fixed 4-week period, so that this case can be handled. The inducer list carries the same SmPC or BNF catch-all as the inhibitor lists. Melatonin from any source is in the CNS depressant exclusion. Over the counter sleep aids are covered by one rule: stopped before the course, with at least 48 hours after promethazine or another long-acting sedating antihistamine. ● Adherence and timing: supply intervals 25 to 35 days (previously 21 to 45); each visit no more than 7 days after the last tablet, and more than 7 nights without tablets ends the course; nights taken out of the last 14 recorded before the response is scored, with at least 10 needed at visit 3; nights withheld for interacting short courses recorded and allowed to extend the interval while keeping visit 3 within 13 weeks. ● Response: adequate response is an ISI fall of 6 or more AND a current ISI of 14 or below AND better daytime functioning AND the patient wishing to continue; an early stop is allowed at visit 2 if the ISI is unchanged or worse and the patient wishes. The source of the threshold is stated (SMC2611 Table 2.2: mean ISI fall at 3 months 7.2 on 50 mg and 5.4 on placebo), with a statement that it is not a published NICE or SMC responder definition. ● CBT-I: "not available" is defined by NHS access, not willingness to pay; free NHS digital CBT-I counts as available where offered (verified: Sleepio free to anyone living in Scotland; SilverCloud Space for Sleep free by self-referral in Wales; Sleepio in England only where locally commissioned); a preference not to try CBT-I is not unsuitability; where no NHS CBT-I is available the pharmacist delivers brief CBT-I components (stimulus control, a sleep window plan, the sleep diary) at supply 1 and reinforces them at visits 2 and 3; the

	patient’s decision on paid CBT-I is recorded. The "does not cover" list now matches the inclusion test. ● Screening: GAD-2 added (GAD-7 if 3 or more; GAD-7 of 10 or more refers) at every visit; Epworth Sleepiness Scale added (above 10 refers); STOP-Bang refers at 5 or more OR 2 or more STOP items with male sex, BMI over 35 or neck over 40 cm, OR any witnessed apnoea; a 1 to 2 week sleep diary is required before supply 1; a Summary Care Record check with consent is mandatory where a record exists, and refusal means no supply. A questionnaire licensing note is added (ISI via the Mapi Research Trust licence; STOP-Bang under its terms) and the ISI item wording is not reproduced. ● GP handover: at supply 1 the patient is told, and it is recorded, that the GP may decline to continue treatment; the visit 3 letter is information plus a request and names a private prescriber as the alternative. ● Consistency: supply 2 is described as at the dose set under Dose and frequency rather than "the same dose"; the body sway finding is attributed to healthy adults under 65, as in SmPC 5.1; the SMC2611 summary now says the European regulator questioned the clinical relevance of the daytime functioning benefits; the TA922 date is corrected to 18 October 2023; records now include allergies, measured BMI and neck, the driving plan, adherence and withheld nights; the 25 mg tablet description from the SmPC is added. ● Exclusions: post-traumatic stress disorder under active treatment; self-harm or a suicide attempt within 2 years (previously 12 months); the sole night-time carer of an infant or dependent adult; any known chronic liver disease; complex sleep behaviour causing or risking injury, or driving while not fully awake, moved to the same-day stop tier. ● Unchanged: ages 18 to 75 at the first supply; ISI 15 or more for entry; PHQ-9 below 10 with question 9 negative at every visit; three numbered supplies, never reset; 25 mg only with a regular moderate CYP3A4 inhibitor; pharmacists only, face to face; one rule per stop symptom, never a dose reduction; short courses of interacting medicines handled by withholding doses, never by changing the dose in the patient’s possession (MPG2 1.1.12); GP registration and notification as conditions of supply.

Appendix 1: Key references

- QUVIVIQ 50 mg film-coated tablets SmPC, Idorsia, PLGB 48711/0003, text revised 12 May 2025, https://www.medicines.org.uk/emc/product/15360/smpc
- QUVIVIQ 25 mg film-coated tablets SmPC, Idorsia, PLGB 48711/0002, text revised 12 May 2025, https://www.medicines.org.uk/emc/product/15359/smpc
- NICE Technology Appraisal TA922, Daridorexant for treating long-term insomnia, published 18 October 2023, https://www.nice.org.uk/guidance/ta922
- Scottish Medicines Consortium, SMC2611, daridorexant (Quviviq), advice 8 March 2024, published 8 April 2024, detailed advice including Table 2.2 (ISI change at months 1 and 3), https://www.scottishmedicines.org.uk/medicines-advice/daridorexant-quviviq-full-smc2611/
- NICE Medicines Practice Guideline 2 (MPG2), Patient Group Directions, recommendations 1.1.1, 1.1.8, 1.1.12, 1.1.14 and 1.5.7, https://www.nice.org.uk/guidance/mpg2
- NICE Clinical Knowledge Summary, Insomnia, https://cks.nice.org.uk/topics/insomnia/
- NICE Medical Technologies Guidance MTG70, Sleepio for treating insomnia and insomnia symptoms, https://www.nice.org.uk/guidance/mtg70
- NHS Grampian, Sleepio and Daylight: free to anyone living in Scotland, funded by the Scottish Government, https://www.gohealthservices.scot.nhs.uk/wellbeing/digital-therapies/ccbt-sleepio-daylight/ (sign-up www.sleepio.com/nhs)
- NHS Wales Online Mental Health Support, SilverCloud Self-Referral Programme Guide (Powys Teaching Health Board): free, self-referral, aged 16 and over living in Wales, including Space for Sleep (based on CBT-I), sign-up nhswales.silvercloudhealth.com
- The Misuse of Drugs Regulations 2001 and amending instruments, legislation.gov.uk (daridorexant not listed).
- British National Formulary, daridorexant, current edition.
- The Human Medicines Regulations 2012, Schedule 16, particulars to be included in a PGD.

Appendix 2: Screening and assessment tools

- **QUESTIONNAIRE LICENSING. The pharmacy uses its own licensed or permitted copies of each questionnaire and does not copy them from this document. The Insomnia Severity Index is used under a licence obtained through the Mapi Research Trust; STOP-Bang is used under its published terms of use; the Epworth Sleepiness Scale is used under the terms set by its copyright holder; PHQ-9, GAD-2 and GAD-7 under their free-use terms. This PGD deliberately does not reproduce the ISI item wording.**
- INSOMNIA SEVERITY INDEX (ISI, Morin): seven items about the last 2 weeks, each scored 0 to 4, total 0 to 28. 0 to 7 no clinically significant insomnia; 8 to 14 subthreshold; 15 to 21 moderate; 22 to 28 severe. Inclusion requires 15 or more at supply 1 (the entry level used in the phase 3 trials). Adequate response at supply visit 3 requires a fall of 6 points or more from the supply 1 score AND a current score of 14 or below (see Dose and frequency for the source). The patient completes it themselves.
- PHQ-9: nine questions about the last 2 weeks, each 0 to 3, total 0 to 27. 0 to 4 minimal; 5 to 9 mild; 10 to 14 moderate; 15 to 19 moderately severe; 20 to 27 severe. Supply requires a total below 10 and question 9 answered "not at all", at every supply visit. The sleep question (question 3) is included in the total as normal.
- GAD-2 AND GAD-7: GAD-2 is the first two GAD-7 questions about the last 2 weeks, each 0 to 3, total 0 to 6. A GAD-2 of 3 or more is followed by the full GAD-7 (total 0 to 21). A GAD-7 of 10 or more excludes and is referred. Done at every supply visit.
- EPWORTH SLEEPINESS SCALE: the chance of dozing in eight everyday situations, each 0 to 3, total 0 to 24. A score above 10 suggests excessive daytime sleepiness, which is not typical of insomnia: exclude and refer.
- STOP-BANG: one point each for Snoring loudly; Tired, fatigued or sleepy in the daytime; Observed to stop breathing, choke or gasp in sleep; high blood Pressure, treated or not; BMI over 35; Age over 50; Neck circumference over 40 cm; male Gender. BMI from height and weight measured in the pharmacy; neck measured with a tape at the level of the Adam’s apple. REFER if: a score of 5 or more; OR 2 or more of the four STOP items (Snoring, Tired, Observed, Pressure) together with male sex, BMI over 35 or neck over 40 cm; OR a "yes" to Observed at any score. STOP-Bang is not used for a patient with diagnosed OSA using CPAP most nights; that patient is assessed under Cautions.
- SLEEP DIARY: each morning for at least 7 and up to 14 consecutive nights before supply 1: time to bed, roughly how long to fall asleep, times and length of waking in the night, final waking time, time out of bed, naps, caffeine and alcohol, and any medicine taken. The diary is continued during the course and reviewed at each visit.

Appendix 3: Sleep hygiene, brief CBT-I components and CBT-I signposting

- SLEEP HYGIENE: keep the same getting-up time every day, including weekends; avoid daytime naps; no caffeine after mid-afternoon; avoid alcohol and nicotine in the evening; regular exercise, but not late in the evening; a cool, dark, quiet bedroom; no screens in bed; a wind-down routine; do not clock-watch.
- **BRIEF CBT-I COMPONENTS, delivered by the pharmacist at supply 1 where NHS CBT-I is not available or is unsuitable, and reinforced at supply visits 2 and 3: (1) STIMULUS CONTROL: go to bed only when sleepy; use the bed for sleep and sex only; if not asleep within about 20 minutes, or awake in the night for that long, get up, do something quiet in low light, and go back when sleepy. (2) A SLEEP WINDOW PLAN: agree a fixed getting-up time every day and a usual bedtime, based on the sleep diary, so that time in bed is not much longer than the time actually spent asleep; review it from the diary at each visit. Formal sleep restriction therapy is not delivered under this PGD. (3) THE SLEEP DIARY, continued through the course. Give the plan in writing.**
- CBT-I SIGNPOSTING: NHS Talking Therapies (self-referral in England) and the GP for local NHS options; free NHS digital CBT-I where offered: Sleepio is free to anyone living in Scotland (www.sleepio.com/nhs); SilverCloud Space for Sleep is free by self-referral for people aged 16 and over living in Wales (nhswales.silvercloudhealth.com); in England, Sleepio (NICE MTG70) where locally commissioned. Paid private face to face or digital CBT-I may be mentioned, and the patient's decision recorded. Record what was offered.

Appendix 4: Interacting medicines

- **STRONG CYP3A4 INHIBITORS, EXCLUDED (contraindicated): itraconazole, ketoconazole, posaconazole, voriconazole, clarithromycin, ritonavir, cobicistat, nirmatrelvir with ritonavir (Paxlovid), idelalisib, and any medicine its SmPC or the BNF describes as a strong CYP3A4 inhibitor. A short course started during treatment: do not take daridorexant from its first dose until 7 days after its last, and record the withheld nights.**
- **MODERATE CYP3A4 INHIBITORS: diltiazem, verapamil, erythromycin, ciprofloxacin, fluconazole, isavuconazole, fluvoxamine, dronedarone, aprepitant, imatinib, crizotinib, letermovir, and any medicine its SmPC or the BNF describes as a moderate CYP3A4 inhibitor. Taken regularly at the first supply: 25 mg. Started regularly during the course: stop daridorexant and return; further supply only at 25 mg if every criterion is met, otherwise refer. A short course during treatment: no daridorexant until 7 days after the last dose, withheld nights recorded. Ciclosporin is also a moderate inhibitor but is excluded below.**
- **CYP3A4 INDUCERS (moderate or strong), EXCLUDED: carbamazepine, phenytoin, phenobarbital, primidone, rifampicin, rifabutin, efavirenz, etravirine, enzalutamide, apalutamide, mitotane, modafinil, bosentan, St John's wort, and any medicine its SmPC or the BNF describes as a moderate or strong CYP3A4 inducer.**
- NARROW THERAPEUTIC INDEX SUBSTRATES, EXCLUDED: tacrolimus, sirolimus, everolimus, ciclosporin, digoxin, dabigatran, colchicine taken regularly, simvastatin 80 mg.
- CNS DEPRESSANTS TAKEN REGULARLY AND CONTINUING, EXCLUDED (a prescribed one is never stopped to allow supply; a non-prescribed sleep aid the patient agrees to stop follows the sleep aid rule below): benzodiazepines; zopiclone, zolpidem; melatonin from any source; mirtazapine, trazodone, tricyclic antidepressants at any dose; antipsychotics; gabapentin, pregabalin; opioids including codeine, dihydrocodeine and tramadol; antiepileptics; sedating antihistamines (promethazine, diphenhydramine, chlorphenamine, hydroxyzine) taken most nights; baclofen, tizanidine; cannabis or cannabis-based products.
- OVER THE COUNTER AND OTHER SLEEP AIDS: the single rule is under Cautions (stopped before the course and not used during it; at least 48 hours after the last dose of promethazine or another long-acting sedating antihistamine before the first dose).
- NO ADJUSTMENT NEEDED (SmPC): hormonal contraceptives, warfarin, rosuvastatin, citalopram, acid-reducing medicines (antacids, H2 blockers, proton pump inhibitors).